

# Designing the PrevMed Onboarding for Seniors 65+: A Research-Backed Playbook

## TL;DR

- For a Medicare-age audience, the biggest gains come from removing friction (fewer fields, phone fallback, larger touch targets) and inserting trust early (visible HIPAA/SOC 2 language, named clinicians, plain-English copy at 6th–8th grade). Rock Health's 2024 Consumer Adoption Survey shows 70%+ of 65+ have used virtual care, but Lam et al. (JAMA Internal Medicine, August 2020) found 38% of older adults (≈13M) cannot complete a self-service video visit — onboarding must assume mixed digital literacy.
- Web-first is the right call: AARP's 2024 telehealth research shows 66% of older adults who don't currently have telehealth would prefer audio-only over video, and Kalicki et al. (J Am Geriatr Soc 2021) report 82% of homebound older adults need caregiver help to complete a telehealth visit — so build proxy/caregiver pathways and an audio fallback from day one, not as v2.
- Insurance verification is the highest-risk drop-off point in any healthcare flow; treat Medicare Beneficiary Identifier (MBI) capture as a "card photo + manual fallback + phone concierge" three-track flow, because Digital Applied's 2026 benchmarks show "Conversion drops modestly from 23.1% at 3 fields to 17.0% at 5 fields, but collapses to 11.4% at 7 fields and 6.9% at 10+ fields," and TAGLAB pegs healthcare patient-registration form abandonment at 20–40%.

## Key Findings

**1. The audience has changed; the design assumptions haven't caught up.** AARP's research insights on health care report that 73% of adults 50+ have used telehealth in the past 12 months, 90% of users were satisfied, and 70% of those 50+ are comfortable with telehealth; Pew (via AARP) finds 76% of those 65+ own a smartphone. Yet Lam et al. (JAMA Intern Med, 2020) found "38% [of older adults] were not ready for video visits, predominantly owing to inexperience with technology," with another "20% of older patients ... unready for telephone visits because of difficulty hearing, difficulty communicating, or dementia." Rock Health's 2024 Survey notes that "11% of Silent Generation consumers haven't tried virtual care because they were uncomfortable or found it too confusing." The target user has the device but not necessarily the confidence — onboarding is where you win or lose them.

**2. Sensory and motor impairment is the baseline, not the edge case.** Using objective measurements from the National Health and Aging Trends Study: 27.8% of adults 71+ have

measured vision impairment (JAMA Ophthalmology 2023); 67.9% of adults 70+ have at least some hearing loss, with 97.6% of those 90+ affected (NHATS Round 11, 2023); 50.4% of adults 65+ have arthritis (CDC MMWR 2023; NCHS Data Brief 497, 2024); mild cognitive impairment reaches 25.2% by ages 80–84 (StatPearls, NIH). Design to WCAG 2.2 AA as a floor and aim closer to AAA on the surfaces seniors actually touch.

**3. The legal floor is rising fast for healthcare.** HHS's Section 504 final rule (May 2024) requires any entity that accepts Medicare/Medicaid funding to meet WCAG 2.1 Level AA by May 11, 2026 (May 2027 for <15 employees). Seyfarth Shaw's ADA Title III tracker reports "Plaintiffs filed 3,117 website accessibility lawsuits in federal court in 2025 — a 27% increase from 2024... 665 more than 2024's total of 2,452." Healthcare is roughly 2–4% of filings (UsableNet 2025) but trending up sharply ahead of the 2026 deadline, with Title III settlements typically \$25K–\$100K plus remediation.

**4. Trust is the single biggest conversion lever, not visual polish.** Rock Health's 2022 report is titled "Moving at the speed of trust." ConversionXL's A/B test data show that adding security badges produced a 12.3% lift in conversions, and Reform.app reports adding real-time social proof alongside trust badges increased conversions by 15% (ConversionXL). Unbounce's healthcare benchmark shows pages written at a 5th–7th grade reading level convert at a 10.8% median, while "messages at the 10th to 12th-grade level reduce the conversion rate by a depressing 55.6%." Unbounce also finds "trust" words ("proven," "recommended," "board-certified") outperform "fear" words ("harmful," "illness") in healthcare.

**5. Insurance verification is the riskiest step.** Per Digital Applied's 2026 benchmarks: "The 5-to-7 cliff is where every added field costs roughly 2.8 points apiece versus 1.5 points before it." TAGLAB pegs healthcare patient-registration abandonment at 20–40%. ConvertCart's documented case shows that making a phone-number field optional reduced abandonment from 39% to 4%. Manual MBI entry is exactly the kind of step where seniors give up: NN/g found "seniors were almost twice as likely to give up on a task" and "when users had problems, seniors blamed themselves 90% of the time, compared to 58% of younger users."

**6. Caregivers are central, but proxy access is poorly built across the industry.** Kalicki et al. (J Am Geriatr Soc 2021) found "82% of homebound patients (mean age 82.7; 46.6% with dementia) required assistance from a family member and/or paid caregiver to complete a telehealth visit." Yet fewer than 3% of caregivers have a designated proxy portal account in most studies, and password-sharing is rampant (Wolff et al., JMIR Aging 2022; Latulipe et al., JMIR 2018). A purpose-built caregiver lane is a category-leading move.

## Details

### Accessibility (vision, motor, cognitive) — the floor

Type, contrast, target sizes.

- Default body text  $\geq 16\text{px}$  (NN/g's senior-targeted floor is 12pt; modern best practice for this audience is 18–20px body).
- Color contrast  $\geq 4.5:1$  for body text and  $\geq 3:1$  for UI components (WCAG 2.2 SC 1.4.3 / 1.4.11). Avoid gray-on-white.
- Touch targets: WCAG 2.2 SC 2.5.8 requires 24×24 CSS pixels minimum (AA); for seniors aim for the AAA standard of 44×44 CSS pixels — matching Apple's Human Interface Guidelines. TestParty notes "the average human fingertip is 16–20mm wide, and users with motor impairments experience error rates up to 75% higher on small targets."
- Provide a built-in text-resize control on the page itself; NN/g observes that most seniors don't know how to use browser zoom.
- Use noncondensed sans-serif, sentence case, generous line height (1.5×). NN/g notes "people read 11% slower for every 20 years they age."

### **Multimodal support.**

- Screen-reader compatible (ARIA labels, semantic HTML, proper focus order).
- Audio-only visit option: AARP found 66% of older adults without telehealth would prefer audio-only.
- Voice input for free-text fields (chief complaint, symptom description); captions and transcripts on all instructional videos.

### **Cognitive load.**

- One question per screen for high-stakes flows (insurance, intake) — Jakob Nielsen's progressive-disclosure principle (NN/g, 1995) directly addresses working-memory limits.
- Avoid disabled buttons; surface specific errors in plain language ("Please enter your date of birth as MM/DD/YYYY") next to the offending field.
- Replace dropdowns with radio buttons where the list is  $\leq 5$  options; Zuko's analytics show dropdowns are uniquely high-friction for users with motor/vision issues.

## **Trust and credibility building**

### **What to show, where:**

- *Above the fold on the landing page:* a real photo of a clinician with name, credentials, and state license; a one-sentence value proposition at 6th-grade level; "HIPAA-compliant" stated in plain English ("Your information is private and protected") plus a SOC 2 or HITRUST badge. Note: there is no official HIPAA logo (Primary Record's "There Is No Official HIPAA Compliance Logo" is a useful counter-reference); pair any badge with a plain-English explanation.
- *At every step that asks for sensitive data:* contextual reassurance ("Only your doctor and care team will see this") plus a "Why we ask" expandable.

- *Provider matching screen*: photo, name, credentials (MD/DO/NP), years of practice, and a 1-line "what I treat." AARP's telehealth report finds 81% of older adults say telehealth "is best with a known health care provider."
- *Social proof*: short, age-matched testimonials with first name + age + state ("Margaret, 72, AZ") rather than influencer content.

### Anti-patterns to avoid:

- Don't restrict trust badges to the footer (Reform: place near submit buttons and sensitive fields).
- Don't use "fear" copy; Unbounce healthcare data shows "harmful" and "illness" depress conversion.
- Don't bury physical address, support phone number, and clinician credentials — NN/g's senior research shows **older users explicitly look for these as legitimacy cues.**

## Reducing drop-off in long flows

### The PrevMed onboarding should be a 5-step funnel, not a single mega-form:

1. **Welcome / value prop** — single CTA "Get started"; optional 30-second captioned explainer.
2. **Account creation** — email + password + name + DOB only (4 fields). **Defer phone number or mark optional** (ConvertCart: 39% → 4% abandonment drop when made optional).
3. **Insurance / Medicare verification** — three-track:
  - **Default**: photo of front/back of Medicare or MA card with auto-OCR via Stedi, pVerify, or Approved Admissions APIs that hit the CMS HETS 270/271 transaction. pVerify and Stedi support automatic Medicare Advantage detection from a "Medicare" card photo.
  - **Manual fallback**: member ID + DOB + name only.
  - **"I'll do this later"**: route to phone concierge or schedule a call. Papa.com and Heyday Health both use phone-first onboarding; Heyday CEO Bobby Shady (Fierce Healthcare, June 2024) attributes adoption to a "Health Ally" who does tech setup for the patient.
4. **Identity verification** — only if clinically required (controlled-substance prescribing, etc.). Use Persona or Mitek with: clear pre-flight ("You'll need your driver's license"), camera permission requested only at this step with a plain-language reason, inline error recovery, gallery-upload alternative.
5. **Provider matching + chief complaint** — 1–3 questions max ("What brings you in today?" with voice and text input; "When would you like to be seen?"; "Do you have a preferred provider?"). **Show the matched provider's photo, name, credentials, and the next available time before requiring final confirmation.**

### Progressive disclosure rules:

- Always show progress ("Step 2 of 5") so users know the end is in sight.
- One concept per screen.
- Defer non-essential fields (emergency contact, full medical history) to the post-first-visit portal.
- Save-and-resume on every step via emailed/texted "pick up where you left off" link.

## **Identity, insurance, and document upload UX**

- Camera capture screens should show a card-sized overlay, auto-detect edges, allow manual capture, accept gallery upload as alternative, and never block on a single low-quality image. Mitek and Kairos recommend "face occupies 60–70% of the frame" instructions with live preview.
- Permissions requests must be just-in-time and explained ("We need your camera to take a photo of your Medicare card. We don't store the photo after we read your member ID.>").
- Inline validation (Lumitech): catch errors at field exit, not at submit. End-of-form rejection is a near-100% abandonment risk for older adults.

## **Caregiver / family involvement**

- Add an explicit **"I'm helping a family member sign up"** entry on the welcome screen — addresses Kalicki's 82% finding head-on.
- Build native proxy accounts (caregiver has their own credentials linked to the patient's record) rather than password-sharing — Wolff et al. (JMIR Aging 2022) and the Bridge/Knack literature flag password-sharing as a HIPAA risk and a privacy hazard, especially for stigmatized conditions (Latulipe et al., JMIR 2018).
- Granular consent: let the patient toggle what the caregiver sees (visit notes yes, billing no).
- After signup, notify the patient that proxy access was granted.

## **Loading states, error recovery, support access**

- Every step exposes a visible "Need help?" link triggering a call-back request or 1-800 number displayed in ≥18pt text — seniors prefer phone for resolution.
- Long-running operations (insurance verification takes 1–5 seconds per Invene's EDI guide) need a friendly progress indicator with reassurance copy ("Checking your Medicare coverage — this usually takes a few seconds").
- Error messages: plain English, specific, with a recovery action. Never "Error 400."
- A persistent chat or callback option that does not disappear at the insurance step (the highest-drop-off point) is the single highest-ROI support feature.

## **Mobile vs. web**

- Web-first is correct for this audience: AARP's 2025 Tech Trends shows 50+ device ownership has grown from 4 to 7 average devices, but desktop/laptop remains the preferred environment for "important" tasks among the 70+ cohort. Zuko's data show desktop completion rates of 47% vs. 42% on mobile.
- Build responsive from the start; do not require an app install for first visit. Heyday's "Health Ally" pattern works in part because tech setup is decoupled from first care.
- When mobile-native arrives, lean on iOS/Android system accessibility features (Dynamic Type, TalkBack) rather than custom controls.

## Medicare / Medicare Advantage specifics

- Distinguish *Original Medicare* (Parts A/B), *MA* (Part C), and *Supplement/Medigap* in plain English ("Original Medicare card is red, white, and blue. Medicare Advantage cards come from private insurers like UnitedHealthcare, Humana, Aetna").
- Use pVerify's MBI lookup or Stedi's Medicare 270/271 to auto-detect MA plan from a "Medicare" card photo — many seniors do not know whether their plan is Original Medicare or MA.
- Comply with the November 8, 2025 CMS requirement to include upstream IP addresses in HETS requests (Stedi docs).
- The May 11, 2026 HHS Section 504 deadline applies to any provider accepting Medicare — accessibility is non-negotiable for this audience.

## Recommendations

### Stage 1 — Ship the MVP onboarding (weeks 0–8):

1. 5-step funnel as described, web-responsive.
2. Insurance verification with photo capture + manual fallback + "call us" option.
3. Plain-language copy reviewed at 6th-grade reading level (Hemingway Editor; NIH Plain Language Action and Information Network resources).
4. WCAG 2.2 AA audit (axe, Lighthouse) + manual screen-reader test.
5. ≥18pt body text, ≥44×44 touch targets, persistent "Need help? Call (XXX)" link.
6. Single named clinician + 3 age-matched testimonials on landing page.
7. Save-and-resume email link.

### Stage 2 — Caregiver lane + audio fallback (weeks 8–16):

1. "Helping a family member" entry path with native proxy accounts and granular consent toggles.
2. Audio-only consultation option exposed at scheduling.
3. Call-back concierge for users who tap "I'll do this later" at insurance verification.
4. Provider matching with photo, credentials, and the next available time before commitment.

### Stage 3 — Optimization (weeks 16+):

1. Voice input for free-text fields (chief complaint).
2. A/B test single-question-per-screen vs. grouped insurance fields.
3. Build per-step drop-off analytics; watch the insurance step like a hawk.
4. Add captioned, age-matched explainer videos at welcome and insurance steps.

### Benchmarks that should change your strategy:

- If insurance-step drop-off > 30%, move to phone-concierge default rather than form default.
- If overall onboarding completion < 45% (Zuko's general baseline), audit field count and reading level first.
- If post-onboarding "did not attend first visit" > 15%, the provider-matching screen is under-selling trust.

### Essential vs. nice-to-have:

| Essential                                          | Nice-to-have                               |
|----------------------------------------------------|--------------------------------------------|
| 6th–8th grade reading level                        | Voice input for forms                      |
| WCAG 2.2 AA compliance                             | AAA (44×44) target sizes                   |
| 44×44 px touch targets                             | Captioned explainer videos at every step   |
| Visible support phone number at every step         | In-app chat                                |
| Plain-English HIPAA explanation + 1 security badge | Animated transitions                       |
| Insurance card photo + OCR + manual fallback       | Auto-detection of MA vs. Original Medicare |
| Save-and-resume                                    | Family member SMS invite                   |
| Named, credentialed provider on landing page       | Provider video introductions               |
| Caregiver/proxy entry path                         | Granular caregiver permission toggles      |
| Audio-only visit option                            | Native iOS/Android apps                    |

### Caveats

- The "38% not ready for video" JAMA figure is from 2018 NHATS data; smartphone adoption among seniors has risen sharply since (Pew via AARP: 76% of 65+ in 2024), so the gap is likely narrower today, but the directional finding holds.

- Conversion benchmarks from Unbounce, Zuko, and Digital Applied are aggregated across industries; healthcare-specific senior conversion data is thin. Treat them as directional; the real benchmark is your first 90 days of instrumentation.
- TAGLAB's "20–40% patient registration abandonment" range is from an aggregator; treat as directional.
- The 16%/22% trust-badge lift figures sometimes cited online could not be verified in primary sources during this research; the confirmed numbers are ConversionXL's 12.3% lift from adding security badges and Reform/ConversionXL's 15% lift from adding real-time social proof alongside trust badges.
- Rock Health's 2024 generational analysis groups 75+ as Silent Generation and 65–74 as Boomers; data for the precise 65+ cohort is not always split out the same way.
- HHS Section 504's May 11, 2026 deadline is binding only on entities that accept federal healthcare funds — confirm Age Bold's specific obligation with counsel, but treat WCAG 2.1 AA (and ideally 2.2 AA) as a hard requirement regardless.
- The Heyday Health, Papa, Galileo, and Devoted Health onboarding patterns are reconstructed from public-facing pages and press; in-app flows may differ. The "Heyday CEO Bobby Shady quote" is from Fierce Healthcare's June 25, 2024 coverage of Heyday's \$12.5M raise.